

RHEUMATOID ARTHRITIS (RA)

PATHOPHYSIOLOGY

- RA is a chronic autoimmune disorder where the immune system mistakenly attacks the synovial membrane (lining of joints), causing inflammation, pain, and joint destruction over time.
- Symmetrical joint involvement (both hands, both knees)
- Affects small joints first (fingers, wrists)

KEY NCLEX CONCEPTS

SYMPTOMS:

- Morning stiffness lasting >30 minutes (often >1 hour)
- Pain, swelling, warmth in joints
- Fatigue, weight loss, fever
- Joint deformities in later stages: ulnar deviation, swan-neck fingers, boutonnière deformity

DIAGNOSIS:

- ↑ ESR (erythrocyte sedimentation rate), CRP (C-reactive protein) → inflammation markers
- Positive rheumatoid factor (RF) or anti-CCP antibodies
- X-rays show joint space narrowing, erosion

TREATMENT

SYMPTOM RELIEF:

- NSAIDs (Non-Steroidal Anti-Inflammatory Drugs) – reduce pain and swelling
(e.g., ibuprofen, naproxen)

DISEASE MODIFICATION:

- DMARDs (Disease-Modifying Anti-Rheumatic Drugs) – slow progression
(e.g., methotrexate, hydroxychloroquine, sulfasalazine)

⚠️ Monitor for bone marrow suppression, liver toxicity, and infection

BIOLOGICS (IF DMARDS INEFFECTIVE):

- TNF inhibitors: etanercept, adalimumab
 - 🧠 Suppress immune system → risk for infection

NURSING INTERVENTIONS

- Encourage rest during flare-ups, balanced with low-impact exercise (e.g., swimming)
- Use warm compresses or showers for stiffness
- Use cold packs for inflammation during acute episodes
- Assistive devices for ADLs (e.g., button hooks)
- Monitor for medication side effects (esp. methotrexate: hepatotoxicity, stomatitis, neutropenia)

NCLEX HIGH-YIELD:

Feature	NCLEX Focus
Morning stiffness	>1 hr = RA (vs OA = <30 min)
Symmetry	RA affects both sides equally
Meds	NSAIDs = relief, DMARDs = slow disease
Labs	↑ ESR, CRP, +RF, +anti-CCP
DMARDs	Immunosuppressive—watch WBC, LFTs